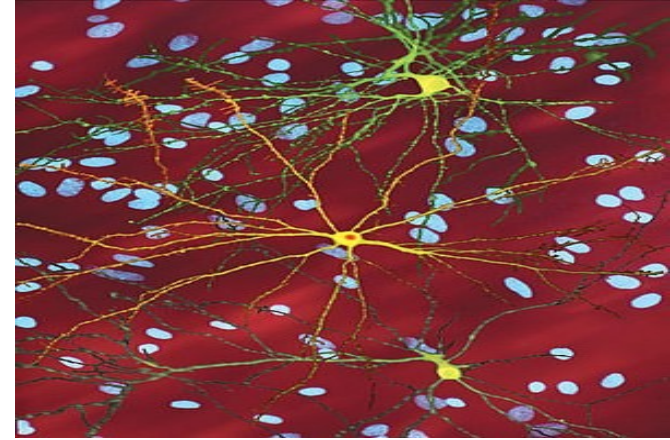


Duty of Care cont.



Duty of Care: The Current Approach (cont.)

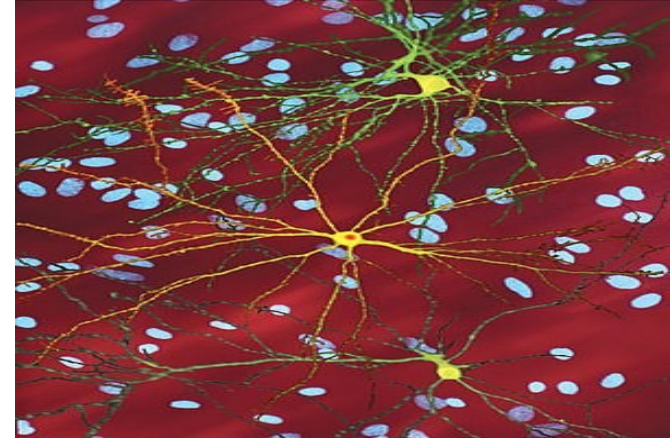
ABC v St George's Healthcare Trust [2020] EWHC 455 (QB)



Facts:

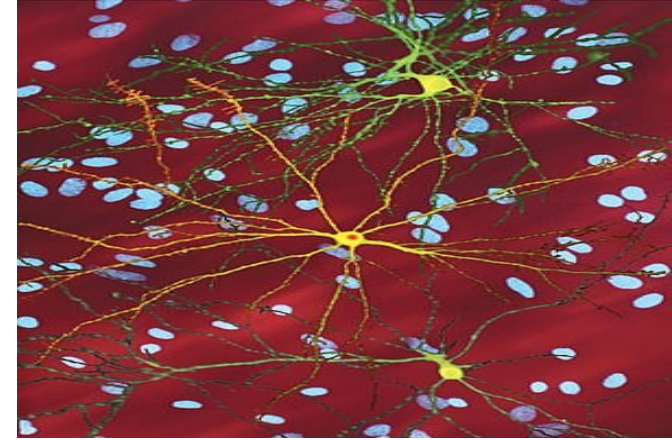
- ❑ Ds' clinicians treating both C and C's father ('XX').
- ❑ Ds learned that C carried the gene for Huntington's disease through treating C's father.
- ❑ C's father did not consent to that information being passed to C, and Ds took the view they should not override their patient's confidentiality.
- ❑ C felt the Ds should have told her about this before it was too late for her to terminate her pregnancy.

ABC v St George's Healthcare Trust [2020] EWHC 455 (QB)



“29. ... the Supreme Court has stressed that there is no general principle capable of providing a practical test applicable in every situation in order to determine whether a duty of care exists and, if so, its scope. ... Rather, the courts must use existing authorities to apply established principles to the facts of the case. It is only in a novel type of case, where established principles do not provide an answer, that the courts need to ask whether a duty of care should be recognised, developing the law incrementally and by analogy with existing authority. The exercise of judgment in those circumstances involves consideration of what is fair, just and reasonable.”

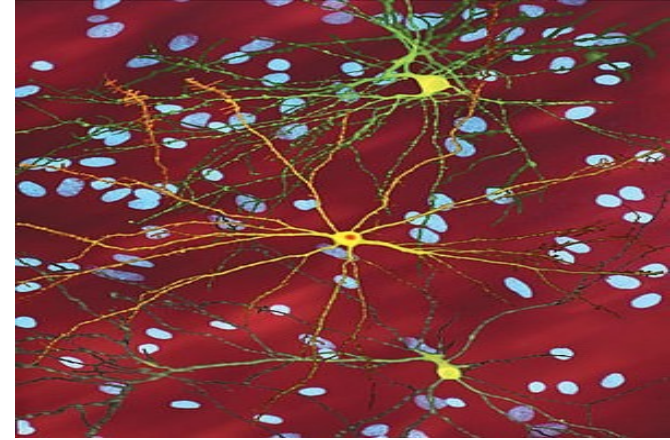
ABC v St George's Healthcare Trust [2020] EWHC 455 (QB)



At trial, C's counsel identified three potential routes to a duty of care [30]:

1. "The claimant was a patient of the defendants ... so that the case falls within the scope of the established duty of care arising out of the doctor-patient relationship.
2. The forensic psychiatry unit ...assumed responsibility for the welfare of the claimant, both in the context of providing family therapy and through her long-standing relationship with the team caring for XX and her involvement in his rehabilitation programme.
3. If neither of the above routes are found to apply, by the application of established principles to the facts of this case by incremental extension ..."

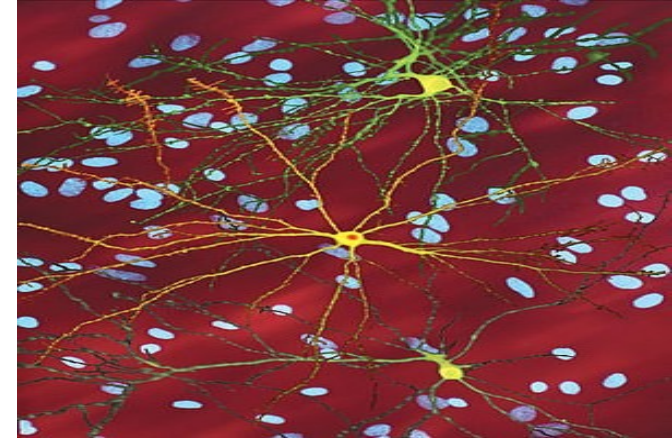
ABC v St George's Healthcare Trust [2020] EWHC 455 (QB)



Case 1: Duty owed as a patient

- ❑ C was a patient of the D's family therapy team. There is an established category of duty: "the duty owed by hospital trusts to patients to deliver their services competently applied to all those undertaking the family therapy." [138]
- ❑ But, that established duty "does not bring with it a right to receive confidential information about other participants" [141]
- ❑ "The duty owed to the claimant in the context of family therapy was quite simply to conduct the therapy with reasonable professional skill and care." [148]

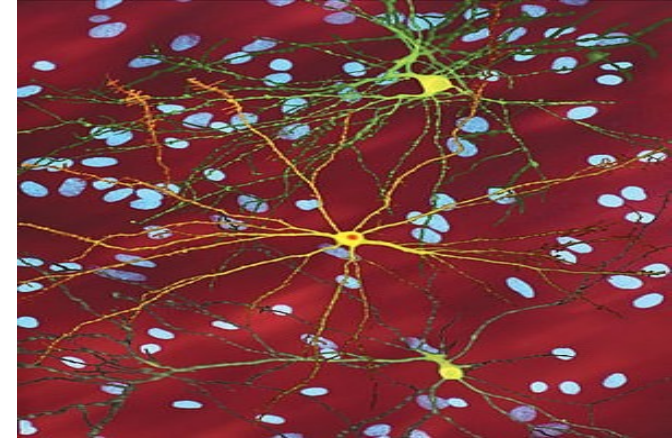
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Case 2: Assumption of responsibility

- ❑ “I do not consider that the claimant can rely upon an assumption of responsibility in the sense required to bring her within the established category of cases where a duty of care exists.” [152]
- ❑ “In my view, this [case] does not involve the application of an established duty to a different factual situation (such as in *Darnley*) but is a novel claim in relation to which a duty of care has not previously been recognised.” [156]

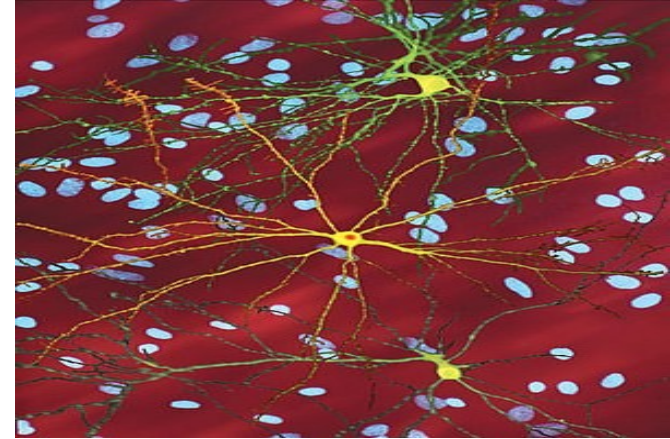
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Case 3: Should the court recognise a duty of care in this novel situation?

- ❑ Duty argued for: “a duty to balance the Claimant’s interest in being informed of her risk of a genetic disorder against her father’s interest in having the confidentiality of that diagnosis preserved.” [158]
- ❑ “[C’s counsel] did not identify any existing authorities in which a doctor or hospital trust had been found to owe a duty to a relative of a patient or to any third person outside the scope of the doctor-patient relationship.” [160]
- ❑ “[D’s counsel] conceded that the existing authorities do not preclude the court finding that a duty is owed by a doctor to a third party.” [161]

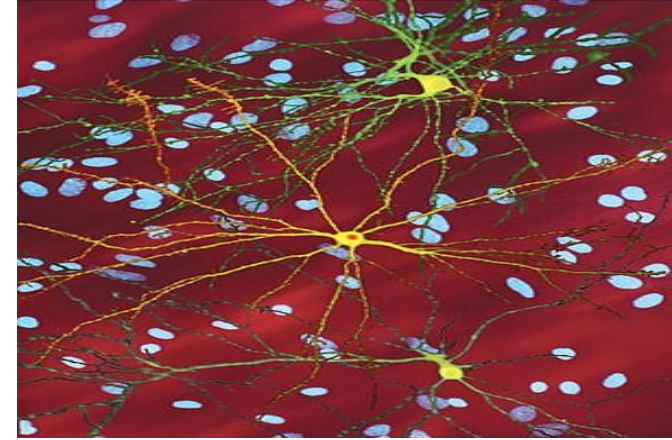
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Step 1 — analogies

- ❑ “the orthodox position is that proximity exists only as between a doctor and his patient and that the doctor’s duty of care does not extend to a third party.” [167]
- ❑ “However ... a doctor often owes duties to more than one person.” [167]
- ❑ “Wrongful birth claims involving failed vasectomies may be seen as further examples of doctors owing duties to more than one person ...” [169]

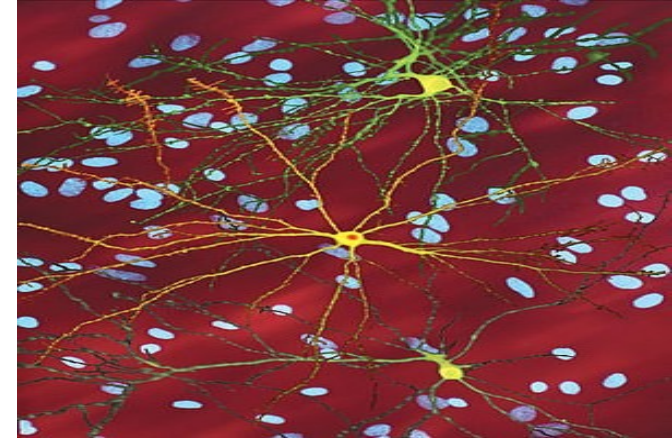
ABC v St George's Healthcare Trust [2020] EWHC 455 (QB)



Step 2 – foreseeability and proximity

- “There was a direct line of communication with the claimant. Had [D] decided to disclose the information to her, the family therapy team offered a route to do so in a supportive environment. In those circumstances, there was a close proximal relationship ...” [173]
- “That the claimant was at risk of suffering harm if the information about her genetic risk was withheld from her was not only foreseeable, it was actually foreseen by [D].” [174]

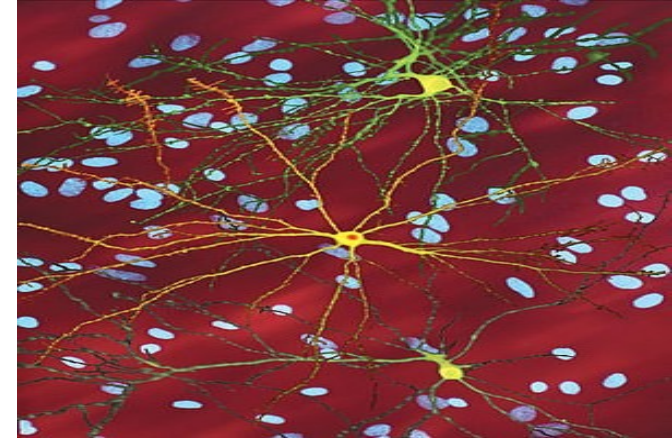
ABC v St George's Healthcare Trust [2020] EWHC 455 (QB)



Step 3 – policy: fairness, justice, and reasonableness

- ❑ “it has long been recognised that the duty of confidence is not absolute.” [178]
- ❑ “I do not consider that a duty to weigh up the claimant’s interests alongside the duty of confidence to XX would damage the relationship of trust and confidence between doctors and patients and/or be detrimental to the public interest.” [179]
- ❑ “I do not accept that recognising a duty to this claimant on the particular facts of this case would open the floodgates to numerous claims. ... [T]here was close proximity between the claimant and [D]. Recognising a duty to her is nowhere near the giant leap that might be required to recognise a duty to multiple relatives around the world.” [182]

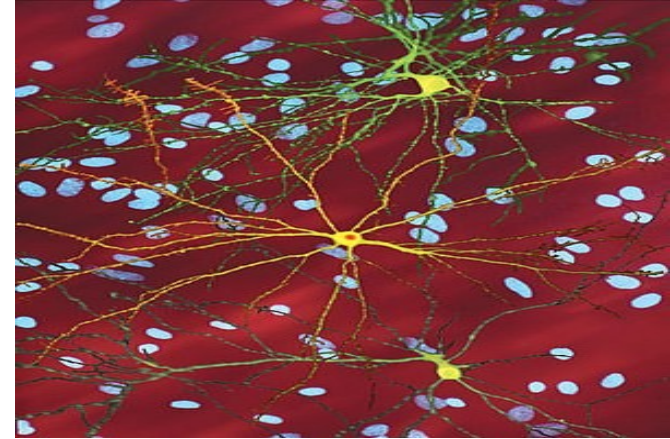
ABC v St George's Healthcare Trust [2020] EWHC 455 (QB)



Step 3 – policy: fairness, justice, and reasonableness

- ❑ “I would readily accept that a duty that distracts medical professionals from the primary care of their patients or which places unreasonable demands on an already stretched National Health Service would be inappropriate. Finding the existence of a duty on the facts of this case would not have that effect.” [183]
- ❑ “recognising a legal duty in circumstances already covered by professional guidance represents only a modest incremental step.” [186]
- ❑ “Article 8 covers both the right to medical confidentiality ... and the right to have information about one’s own health. ... Recognising a common law duty to both parties to conduct a proper balancing exercise in accordance with the professional guidance is consistent with the way in which the law has developed to take account of the Convention.” [187]

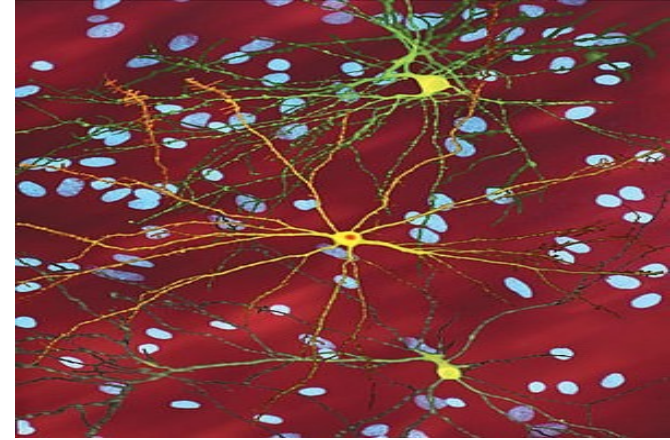
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Conclusion on duty of care

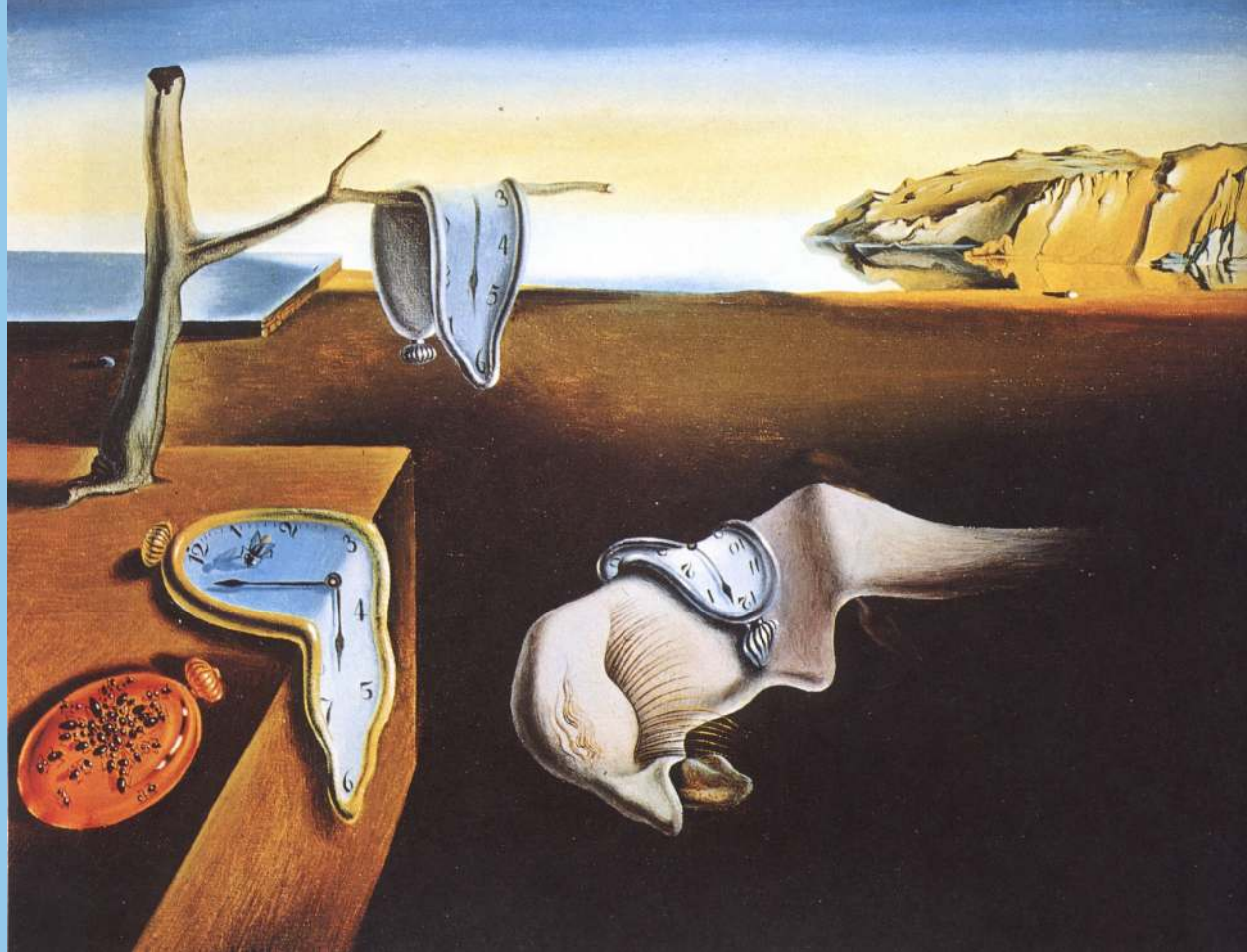
“I conclude that it is fair, just and reasonable to impose on [D] a legal duty to the claimant to balance her interest in being informed of her genetic risk against her father’s interest in preserving confidentiality in relation to his diagnosis and the public interest in maintaining medical confidentiality generally. The scope of the duty extends not only to conducting the necessary balancing exercise but also to acting in accordance with its outcome.” [188-189]

ABC v St George's Healthcare Trust [2020] EWHC 455 (QB)



Held:

- ❑ No breach of duty: “the decision not to disclose was supported by a responsible body of medical opinion and cannot be considered to have amounted to a breach of the duty I have identified.” [231]
- ❑ No causation: “the claimant has not proved that she would have undergone a termination if notified of the risk during pregnancy.” [253]



Duty of Care: The History

The early days



- ❑ Negligence was ‘fragmented’ – a series of specific duties in specific contexts
 - *Gladwell v Steggall* (1839) – Duty of doctor to patient
 - *Smith v Baker & Sons* (1891) – Duty of employer to employee
 - *Blacker v Lake & Elliot* (1912) – D not liable when he sold product to A which then, because of defect, injured C
- ❑ “It is remarkable how difficult it is to find in the English authorities statements of general application defining the relations between parties that give rise to the duty. The Courts are concerned with the particular relations which come before them in actual litigation, and it is sufficient to say whether the duty exists in those circumstances.”
(Lord Atkin, in *Donoghue*)

The fundamental breakthrough: *Donoghue*



“You must take reasonable care to avoid acts or omissions which you can reasonably foresee would be likely to injure your neighbour. Who, then, in law is my neighbour? The answer seems to be — persons who are so closely and directly affected by my act that I ought reasonably to have them in contemplation as being so affected when I am directing my mind to the acts or omissions which are called in question.”

(Lord Atkin)

The fundamental breakthrough: *Donoghue*



The dissents:

- “the common law ... though its principles are capable of application to meet new conditions not contemplated when the law was laid down ... cannot be changed nor can additions be made to them because any particular meritorious case seems outside their ambit.” (Lord Buckmaster)
- “If one step, why not fifty?” (Lord Buckmaster)

The 'established category' from *Donoghue*



To this day, *Donoghue* and the cases immediately after it stand as good authority for the following propositions:

- ☐ You have duty not to do negligent acts which will reasonably foreseeably cause physical injury to others.
- ☐ You have duty not to do negligent acts which will reasonably foreseeably cause damage to others' property.

The era of expansion



“[W]hen a new point emerges, one should ask not whether it is covered by authority but whether recognised principles apply to it. *Donoghue* ... may be regarded as a milestone, and the well known passage in Lord Atkin's speech should I think be regarded as a statement of principle. ...[T]he time has come when we can and should say that it ought to apply unless there is some justification or valid explanation for its exclusion.”

(*Dorset Yacht Co Ltd v Home Office* [1970] UKHL 2, per Lord Reid, p.1026–1027)

The era of expansion



The “two stage test” from *Anns v Merton LBC* [1977] UKHL 4:

1. Could D reasonably foresee that their negligence would cause damage to C? If ‘yes’, then a prima facie duty of care arises.
2. If yes, are there any considerations which negate, reduce, or limit the scope of the duty? If ‘no’, then there is a duty.

The era of expansion



- ❑ The high-water mark: *Junior Books Ltd v Veitchi Co Ltd* [1983] 1 AC 520 — duty of care extended to pure economic loss arising from defective property!

But, everyone (mostly) agreed things had gone too far:

- ❑ *Anns* overruled by *Caparo* in 1990
- ❑ *Junior Books* overruled in 1991 by *Murphy Brentwood DC*

Reining it in: *Caparo v Dickman* [1990] 2 AC 605



- “in addition to the foreseeability of damage, necessary ingredients in any situation giving rise to a duty of care are that there should exist between the party owing the duty and the party to whom it is owed a relationship characterised by the law as one of 'proximity' or 'neighbourhood' and that the situation should be one in which the court considers it fair, just and reasonable that the law should impose a duty of a given scope upon the one party for the benefit of the other.” (Lord Bridge)

Reining it in: *Caparo v Dickman* [1990] 2 AC 605



- ❑ But *Caparo* did not intend to create a “general test”!
- ❑ “[Foreseeability, proximity, and fairness, justice & reasonableness] amount in effect to little more than convenient labels to attach to the features of different specific situations which, on a detailed examination of all the circumstances, the law recognises pragmatically as giving rise to a duty of care ...” (Lord Bridge)
- ❑ “the law should develop novel categories of negligence incrementally and by analogy with established categories ...” (Lord Bridge, citing Brennan J. in the High Ct of Australia)



Misunderstanding *Caparo*: courts love tests

Over 100 uses of the phrase “the *Caparo* test” before 2018.
E.g.:

- ❑ *Robinson* in the Court of Appeal (now overruled) — “the *Caparo* test applies to all claims in the modern law of negligence”. (Hallet LJ, [40])
- ❑ Many references to the three-part “*Caparo* test” — foreseeability, proximity, and fair, just and reasonable

Trying to get back to incrementalism



“In *Caparo* ... Lord Bridge emphasised the inability of any single general principle to provide a practical test which could be applied to every situation to determine whether a duty of care is owed and, if so, what is its scope. ... Paradoxically, this passage in Lord Bridge’s speech has sometimes come to be treated as a blueprint for deciding cases, despite the pains which the author took to make clear that it was not intended to be any such thing.”

(*Michael v CC of South Wales* [2015] UKSC 2, Lord Toulson, [106])

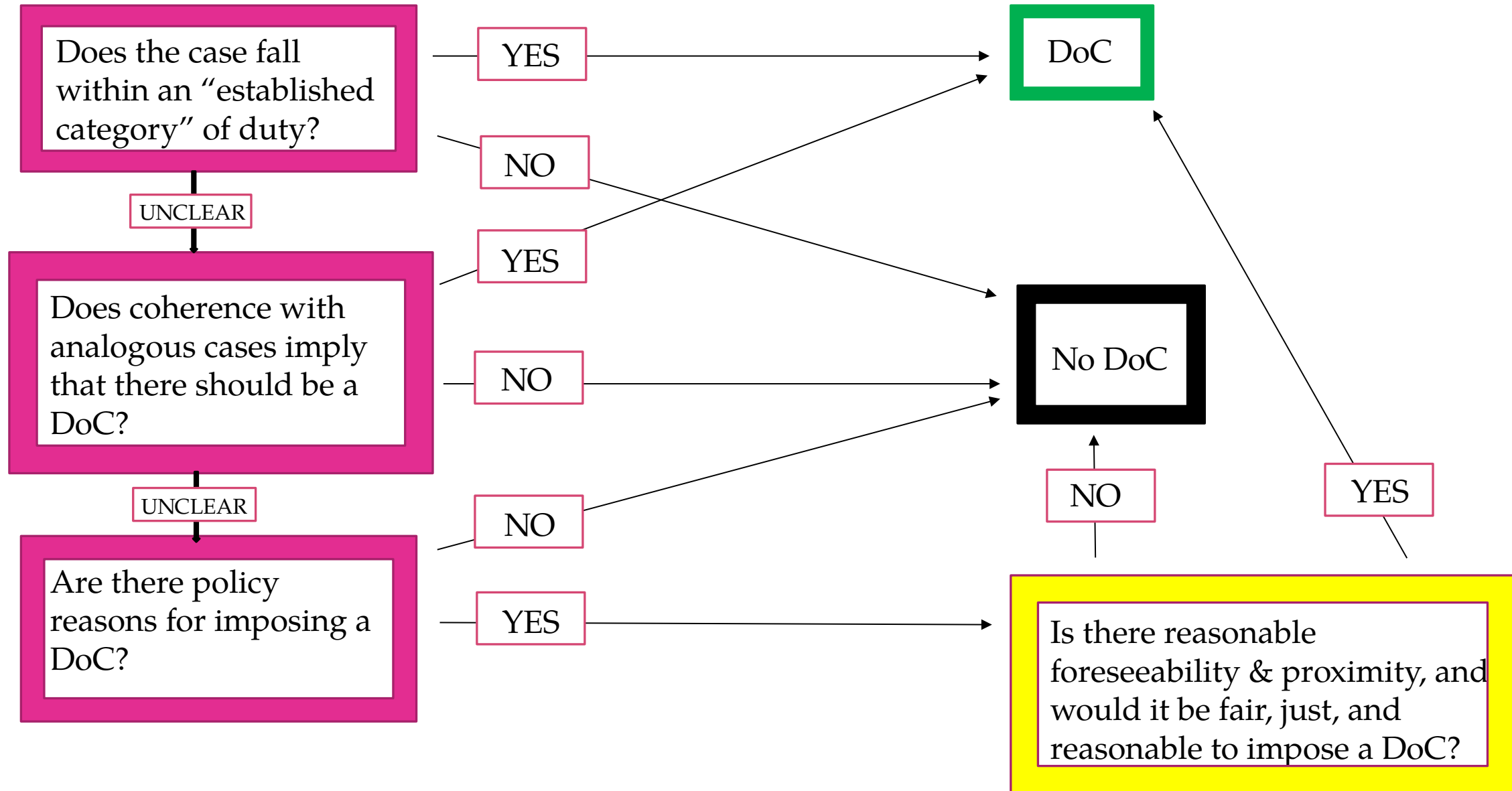
The current approach (again): *Robinson*



“The proposition that there is a *Caparo* test which applies to all claims in the modern law of negligence, and that in consequence the court will only impose a duty of care where it considers it fair, just and reasonable to do so on the particular facts, is mistaken ... [T]he whole point of the *Caparo* case ... was to repudiate the idea that there is a single test which can be applied in all cases in order to determine whether a duty of care exists, and instead to adopt an approach based, in the manner characteristic of the common law, on precedent, and on the development of the law incrementally and by analogy with established authorities.”

(Lord Reed, [21])

Establishing a Duty of Care — Flowchart



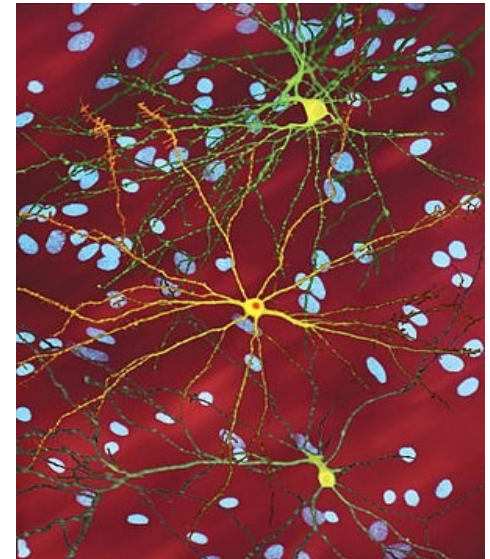
Affirming *Robinson: N v Poole BC* [2019] UKSC 25



“[Robinson] explained, as *Michael* had previously done, that *Caparo* did not impose a universal tripartite test for the existence of a duty of care, but recommended an incremental approach to novel situations, based on the use of established categories of liability as guides, by analogy, to the existence and scope of a duty of care in cases which fall outside them. The question whether the imposition of a duty of care would be fair, just and reasonable forms part of the assessment of whether such an incremental step ought to be taken. It follows that, in the ordinary run of cases, courts should apply established principles of law, rather than basing their decisions on their assessment of the requirements of public policy.” [64]

Applying *Robinson*

- *Darnley* shows you how it works for cases which are inside an “established category” — and how we define the breadth of those categories
- *ABC* shows you how it works for cases which do not fall within established categories, where policy reasoning must be applied



Back to the early days?



Does all of this take us back to the original position, where the courts focused on specific duties in specific contexts, rather than general principles? Not quite:

- ❑ The era of expansion means that there are now many more established categories than there were in the early days.
- ❑ In the early days it was just established categories, and no general principle. Now we have a mix: we begin with established categories, and turn to principle where things are unclear.